Archived

The Clinician’s AI Safety Check Version 1.0, May 2026

This version was superseded by version 2.0 in November 2026.
It is kept as a record of what was published.
Don’t use it for practical guidance.

Use the current version:
clinicallyintelligent.com/field-guides/safety-check

What changed in version 2.0:

- A specific outcome for each answer, instead of “answer yes to every question”.
- No composite patient described as safe: there’s no safe number of details.
- Starts by asking whether the tool, account and use are approved.
- After a mistake: stop, report, record, contain, cooperate. Not “delete the chat”.
- No “30 second” promise, and no blanket statements about consumer tools.
- All nine checks, including the missing Check 3.

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CLINICALLY INTELLIGENT

FIELD GUIDE 01
AI SAFETY PLAYBOOK

The Clinician's AI Safety Check



The 30 second check clinicians should run before using AI at work.

A three stage, nine question framework for NHS clinicians using consumer AI tools in clinical work.

Run this check every time.
30 seconds. Then proceed — or do not.

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This is a check, not a guide.

Run it before each use — not once at the start.

The framework has three stages:

01 **Before using the tool**

02 **While using the tool**

03 **After using the tool**

Each stage contains three questions. You must answer yes to every question before proceeding.

If you cannot answer yes:

- ✕ **Do not use the tool yet**
- ! **Use the tool differently**
- ? **Ask your IG lead before proceeding**

IMPORTANT

Consumer AI tools — including ChatGPT, Perplexity, Claude, Otter, and Magpie — are not NHS-procured systems.

They have not been assessed against:

- DCB0129
- DCB0160

Treat anything you upload as potentially used for AI training somewhere in the chain.

Composite scenarios.
Unsafe input.

Real patient information.

A composite scenario is:

"An older adult several weeks post-stroke living at home with a spouse."

Note:
"Mrs Smith from yesterday."

When in doubt, ask your IG lead.

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Before your open the tool

Ask these three questions before opening any AI tool.

CHECK 1

Have I changed the patient or scenario into a composite?

A composite describes a patient type — not a real person.

IDENTIFIABLE (Do not use)

Mrs S

(Not based on a real patient)



- 83 years old
- Stroke 3 weeks ago
- Lives alone in Stratford
- Seen yesterday afternoon
- Referred by Dr A. Khan
- Newham Hospital, Ward B
- Mobilises with frame, 1-person assist

→

COMPOSITE (Safe to use)

Mrs S Composite

(Protected scenario)



- Older adult (80s)
- Several weeks post-stroke
- Lives alone at home
- Recent clinical review
- Under care of multidisciplinary team
- Acute hospital setting
- Mobilises with walking aid, needs supervision

CHECK 2

Have I removed every identifying combination?

Single details rarely identify a patient. Combinations do.

Age + condition + area + living situation can narrow to one person in your service.

Stay at the population level:

- older adult
- several weeks
- post-stroke
- lives alone at home

Combinations identify patients — not single details.

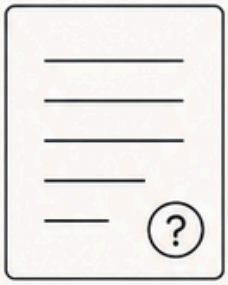
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While you are using the tool

Ask these three questions while using the AI tool.

CHECK 4

Am I keeping the scenario at composite level, even when the AI asks for more detail?



AI tools often ask follow-up questions that push toward identification.

Examples:

> "What is the patient's name?"

> "Which borough do they live in?"

> "What is their exact age?"

Leave these fields blank or use placeholders.

CHECK 5

Has every citation, guideline, and factual claim been verified?

AI tools:

- invent citations
- misquote sources
- provide broken links
- generate confidently incorrect information

Before using any output clinically:

- ✓ Click every link
- ✓ Check every citation
- ✓ Verify every claim against the original source

AI confidence is not evidence.

CHECK 6

If I realise I've made a mistake, do I know what to do?

If you accidentally post identifiable information:

1. Stop the session immediately
2. Delete the chat / public
3. Note what happened and which tool was used
4. Tell your IG lead today

Mistakes that are reported get managed.

Mistakes that are hidden become incidents.

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After you have used the tool

Ask these three questions before saving, sharing, or using the output.

CHECK 7

Is this output for one specific patient I have assessed?

If yes, this usually sits within normal clinical practice — provided your Trust permits the AI workflow used to create it.

Examples:

- a written home exercise programme
- a personalised energy conservation plan
- ADL recommendations after a home visit

Personalise the output inside your Trust documentation systems.

Date and version every copy.

At the point of personalisation, it becomes part of the clinical record.

CHECK 8

Is this output intended for multiple patients or wider distribution?

If the material will be:

- reused
- displayed on a ward
- shared externally
- distributed across a service

... it enters a different governance category.

This usually sits under:

- Patient Information Group
- Communications Governance review process

Examples:

- Posters in clinical areas
- Service overview leaflets
- Patient surveys

CHECK 9

Have I dated, versioned, and filed the output correctly?

Keep three separate:

Library entries
Reusable templates built from composite scenarios.

Store in:
- personal workflow library
- service AI resource folder

Clinical records
Patient-specific outputs.

Store in:
- Trust documentation systems
- EPR

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Questions your IG lead can answer

Most clinicians do not know who their IG lead is until they urgently need them.



Ask your line manager

They can tell you who your IG lead is and what local process to follow.



Check your Trust intranet

Look for IG policies, AI guidance, and approved tools for clinical use.



Find organisation-wide guidance

Look for guidance on data protection, AI use, and reporting.

Questions worth asking

- Is my Trust permissive or restrictive on consumer AI tools?
- Has my Trust procured any approved AI tools?
- Are there approved use cases I'm unaware of?
- What is the reporting process if I think I've made a mistake?
- Where should AI-assisted guidance be stored?

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If something goes wrong

Most instinct is to hide the mistake.
Do the opposite.

1. **Stop**
   End the session immediately.

2. **Note**
   Write down:
   - what was pasted
   - when it happened
   - which tool was used

3. **Tell**
   Inform your IG lead today.

4. **Cooperate**
   Follow the process they recommend.

5. **Document**
   Record what happened and what advice was given.



The profession is more forgiving of reported mistakes than hidden ones.

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This document is general professional guidance only.

It is not a substitute for:

- your Trust / information governance policies
- local clinical governance guidance
- advice from your IG lead

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The AI tools referenced in this document are consumer products. They have not been independently assessed by the author against DCB0129 or DCB0160 clinical risk management standards and may not be approved for clinical use by your employer.

Before applying any workflow described in this document to your own practice, you are responsible for ensuring that its use is permitted under your Trust information governance policies, DCB0129 or DCB0160 obligations, relevant regulation, and any applicable local governance or contractual restrictions.

Patient identifiable information should not be entered into consumer AI tools unless explicitly permitted within an approved organisational workflow or sanctioned environment.

Application of any workflow described requires your own professional judgement, local policy compliance, and Trust sanctioning where applicable.

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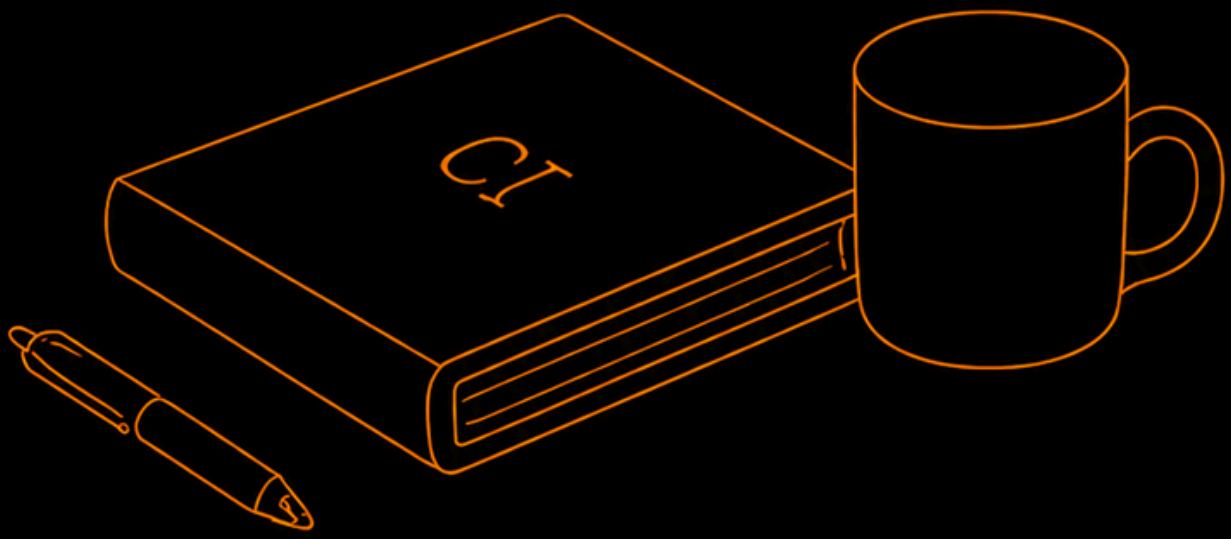
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Healthcare is changing.
Some clinicians will wait.
Others will help shape what comes next.

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